

1. Administrative & Study Identification

Brief Study Title: A Study of Acalabrutinib Plus Venetoclax and Rituximab in Participants With Treatment Naïve Mantle Cell Lymphoma **Official Full Study Title:** A Multicentre, Phase II, Open-label Study to Evaluate the Efficacy of Acalabrutinib in Combination with Venetoclax and Rituximab in Participants with Treatment Naïve Mantle Cell Lymphoma (TrAVeRse) **Short Title/ Acronym:** TrAVeRse **Protocol Number:** D822GC00001 **NCT Number:** NCT05951959 **Sponsor/Company Name:** AstraZeneca **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING) **Investigational Product:** Acalabrutinib, Venetoclax, and Rituximab (AVR) **Phase of Development:** Phase II **Medical Monitor:** AstraZeneca Clinical Operations Lead / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To estimate the efficacy of the AVR combination by assessing the Complete Response (CR) rate with minimal residual disease (MRD) negativity in participants with treatment-naïve Mantle Cell Lymphoma (MCL). * **Secondary Objectives:** To assess the duration of response (DoR), time to next therapy (TTNT), event-free survival (EFS), overall survival (OS), and the feasibility of response-adapted treatment cessation for participants achieving an MRD-negative CR (observation vs. acalabrutinib maintenance).

2.2 Background & Rationale Despite advances in the treatment of MCL, standard chemoimmunotherapy remains the primary option but is associated with severe long-term toxicities. This study evaluates a chemotherapy-free, targeted triplet combination regimen (AVR: Bruton's Tyrosine Kinase inhibitor + BCL2 inhibitor + anti-CD20 monoclonal antibody) to effectively treat MCL while avoiding traditional chemotherapy side effects.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Active Comparator / Observation (in the maintenance phase) * **Blinding:** Open-label * **Randomization:** 1:1 post-induction assignment (for participants who achieve MRD-negative CR at the end of Cycle 13, randomized to acalabrutinib maintenance vs. observation).

3.2 Planned Enrollment & Duration * **Targeted Enrollment:** 108 participants * **Number of Sites:** Multicenter, International

(Cancer centers in the US, UK, Australia, Canada, and Europe) *
Estimated Study Start: 13-Dec-2023 * **Estimated Primary Completion:** 07-Jul-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years or the legal age of consent. 2. Histologically documented MCL based on WHO criteria, with confirmed chromosomal translocation t(11;14)(q13;q32) and/or overexpression of cyclin D1. 3. Clinical Stage II, III, or IV requiring systemic treatment, with no prior therapies for MCL, and at least 1 measurable site of disease per Lugano Classification.

4.2 Exclusion Criteria 1. Active central nervous system (CNS) involvement by lymphoma or leptomeningeal disease. 2. Active infection with HIV, Hepatitis B (HBsAg positive or HBV-DNA PCR positive), or Hepatitis C (HCV-PCR positive). 3. Clinically significant cardiovascular disease (e.g., uncontrolled arrhythmias, heart failure, myocardial infarction within 6 months of screening, or QTc > 480 msec).

5. Intervention & Treatment Plan

5.1 Investigational Regimen * Dosage/Frequency & Agent Details: * **Acalabrutinib:** 100 mg twice daily (BID) orally, starting Cycle 1, Day 1 for 13 cycles. * **Venetoclax:** Once daily (QD) orally, starting Cycle 2, Day 1 for 12 cycles (with a 5-week dose ramp-up to a maximum of 400 mg). * **Rituximab:** Intravenous (IV) infusion on Day 1 of every cycle, for 12 cycles. **ATC Code:** L01FA01. **Trade Names:** Rituxan, Riabni, Ruxience, Truxima, Rituxan Hycela. (EMA Information: [Ruxience EPAR](#) | [Truxima EPAR](#)) * **Route of Administration:** Oral (Acalabrutinib, Venetoclax) and Intravenous (Rituximab). * **Duration of Treatment:** 13 induction cycles (28 days per cycle), followed by 1 cycle of acalabrutinib alone, then response-adapted maintenance (acalabrutinib or observation). Total study duration is approximately 67 months.

5.2 Concomitant & Prohibited Therapy * Permitted: Short courses of glucocorticoids (>20 mg prednisone for ≤ 14 days) for comorbid conditions; systemic corticosteroids up to 100 mg daily for up to 10 days as pre-phase control. * **Prohibited:** Live attenuated vaccines within 28 days of Day 1, strong CYP3A inhibitors/inducers, warfarin, and concurrent participation in other therapeutic trials.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent,

Baseline Scans, Serology, Cardiac evaluation | | **Baseline** | Day 1
| Cycle 1 Day 1 dosing (Acalabrutinib + Rituximab initiation) | |
Interim | Monthly | Monthly cycles 1-13 (Venetoclax ramp-up
starts Cycle 2). MRD blood sampling at C1-C4, C7, C10. Post-
induction visits Q3M (Year 1) then Q6M. | | **End of Study** | Month
67 | Final disease assessment, survival tracking, exit interview,
and IP accountability |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * **Definition:** Measurable residual disease (MRD)-negative complete response (CR) rate at the end of the AVR induction phase (end of Cycle 13). * **Measurement Method:** Evaluated using central laboratory MRD analysis from peripheral blood/bone marrow combined with investigator-assessed tumor response per the Lugano Classification.

7.2 Secondary & Exploratory Endpoints * [Endpoint 1] Overall Response Rate (ORR), Duration of Response (DoR), Progression-Free Survival (PFS), and Event-Free Survival (EFS). * [Endpoint 2] Time to Next Therapy (TTNT) or death from any cause (Overall Survival).

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Continuous clinical and lab monitoring, with specific attention to bleeding risks, cardiovascular events (e.g., atrial fibrillation), and Tumor Lysis Syndrome (TLS) during venetoclax ramp-up.
 - **Serious Adverse Events (SAEs):** Must be reported to the sponsor and regulatory authorities within 24 hours of site awareness.
 - **Data Monitoring Committee (DMC):** Safety and tolerability evaluated periodically by an independent Data Monitoring Committee.
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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy outcomes; Safety Set for all participants receiving at least one dose of study medication.
- **Sample Size Justification:** A targeted enrollment of 108 participants provides adequate statistical power ($\alpha = 0.05$, $\text{Power} \geq 80\%$) to robustly estimate the MRD-negative CR rate of the targeted triplet regimen in the first-line setting.
- **Handling of Missing Data:** Missing MRD assessments at the primary endpoint evaluation will generally be imputed as MRD-positive (non-responders) per standard hematology trial SAP guidelines.

10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
 - **Monitoring Plan:** Monthly onsite/remote monitoring during the induction phase, transitioning to quarterly/semi-annual during the maintenance phase.
 - **Audit Trail:** Utilization of a validated Electronic Data Capture (EDC) system with strict eCRF locking mechanisms and audit trails.
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11. Study Identifiers & Governance

- **Posting ID:** 338616605240275901
 - **Primary ID:** D822GC00001
 - **Registry Number:** NCT05951959
 - **Sponsor/Lead Organization:** AstraZeneca
 - **Study Acronym:** TrAVeRse
 - **Official Regulatory Title:** A Multicentre, Phase II, Open-label Study to Evaluate the Efficacy of Acalabrutinib in Combination with Venetoclax and Rituximab in Participants with Treatment Naïve Mantle Cell Lymphoma (TrAVeRse)
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12. Clinical Framework (Mantle Cell Lymphoma Specific)

- **Primary Condition / Disease Area:** Mantle Cell Lymphoma (MCL) / Lymphoma
 - **Preferred UMLS Name:** Mantle cell lymphoma
 - **Semantic Type:** Neoplastic Process
 - **Disease Definition:** A cancer originating in lymphocytes and presenting as a solid tumor of lymphoid cells.
 - **Ontology Codes:**
 - **MeSH Code:** D008223
 - **HPO Code:** HP:0002665
 - **SNOMED ID:** 118600007
 - **Diagnosis Threshold:** Histological evidence with documented t(11;14)(q13;q32) translocation and/or cyclin D1 overexpression
 - **Medical Methodology:** Chemotherapy-free triplet induction regimen (AVR)
 - **Optimization Target:** Achievement of an MRD-negative Complete Response (CR)
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13. Study Procedures & Methodology

- **Management Pathway:** 13-cycle AVR targeted induction, followed by response-adapted maintenance (acalabrutinib continuation vs. observation).
 - **Education Protocol (HCP):** Site initiation visits and explicit training on Venetoclax TLS risk mitigation/ramp-up protocols.
 - **Education Protocol (Patient):** Onsite training for self-administering oral agents (Acalabrutinib, Venetoclax), tracking doses, and monitoring for side effects.
 - **Quality Audit Mechanism:** Centralized laboratory MRD processing, independent response review, and regular source data verification.
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14. Observation & Follow-Up Schedule

- **Total Duration:** Up to 67 months (Approx. 5.5 years)
 - **Onsite Visit Cadence:** Every 28 days for Cycles 1-14; thereafter, every 3 months (Q3M) for Year 1, and every 6 months (Q6M) subsequently.
 - **Remote Monitoring Frequency:** Intermittent phone screening between long-term follow-up visits to track disease progression and subsequent therapies.
 - **Checklist Review Interval:** End of Cycle 13 (Day 1 of Cycle 14) for definitive response assessment and randomization.
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15. Sign-off & Approval

	Role		Name		Signature		Date			:---		:---		:---		:---	
	Principal Investigator				[Insert Name]			_____		[DD-MMM-YYYY]							
	Clinical Operations Lead				[Insert Name]			_____		[DD-MMM-YYYY]							
	Lead Statistician				[Insert Name]			_____		[DD-MMM-YYYY]							